[date]

Professor Lawrence F. Katz
Editor, *The Quarterly Journal of Economics*

Dear Professor Katz,

In [month year] the Journal declined my paper "Housing First or Treatment First?" ([manuscript number]). I have since written a different paper, "Children First, Housing Later," and would like to ask whether you would consider it as a new submission. It builds on the same household model of drug use, housing and homelessness, but its question, its mechanism, its experiments and its conclusion are new.

The question and the answer. The first paper asked whether conditioning housing assistance on sobriety reduces addiction and homelessness through moral hazard. It found that, holding the number of vouchers fixed, conditionality lowers addiction but raises homelessness, and it favoured Treatment First only when combined with universal vouchers. The new paper shows that moral hazard does not decide the question: without effects on children, a newborn is worse off under Treatment First, by 1.09 percent of lifetime consumption, even though drug use falls. The answer turns on the children. A parent's drug use costs the child consumption and housing at home, and earnings as an adult, and the parent counts these only in part. Within the existing rationed programme, Treatment First cuts drug use by 21 percent and homelessness by 11 percent, and raises a newborn's welfare by 2.57 percent, three quarters of it an externality the parent does not count.

The model. Its new elements are children with imperfectly altruistic parents and an earnings penalty from a parent's disorder, serious mental illness, a choice of treatment, a drug test whose deterrence is disciplined by the US military's random testing, a drug price that clears its market (it was fixed in the first paper), and a budget balanced through the rationing of vouchers rather than through taxes.

The transition. In the transition, which covers every cohort alive at the reform and clears the drug price and the budget every period, homelessness and childhood homelessness never rise, and children born at the reform gain 1.45 percent.

The trials. The model reproduces the micro evidence of randomised controlled trials, including that households with a disorder lose from the condition. What reverses the verdict is what trials cannot observe: the households the condition deters from using, the vouchers its savings fund, and the children.

The question is current: in July 2025 an executive order directed federal agencies to end support for Housing First. I attach the new paper and would be grateful for your guidance.

Sincerely,

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